

India has a billion patients and no front door to care.

Iyashi

Care that reaches you.

India's single front door to healthcare, built around one thing done right: a real doctor, on demand, at your door. The emergency button brings people in. Medicine delivery comes next.

The doctor · today The button · the hook Delivery · next

India improvises.

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THE PROBLEM

02

There is no single front door to healthcare. In the moment that matters, three questions have no good answer:

Which number?

100, 102, 108, a private ambulance. No one remembers, no one knows who is nearest.

Which doctor?

Days in a waiting room, or a video call with a stranger who cannot examine you.

Which app?

Diagnostics here, pharmacy there, records nowhere.

1:1,511 people per government doctor
against the WHO norm of 1:1,000

India: 66% of the norm

WHO 1:1,000

~70% of specialists sit in cities
leaving rural India to travel for basic care

400M+ Indians with no health insurance
one unexpected bill from crisis

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PROBLEM · 02 / 12

One product, done right, then we widen.

We don't try to be three companies at once. We win the front door with the highest-trust, highest-frequency moment first, a doctor at home, and earn the right to the rest.

TODAY · OUR CORE

The doctor at your door

On-demand doctors to your home, sorted by distance, price and rating. A video consult now, or a real in-person visit, with your records travelling with you. This is the business.

THE HOOK

The button

In a crisis, one tap shares your location and puts the nearest doctor on alert, the reason people install lyashi before they ever need a consult.

NEXT

Delivery

Once the doctor relationship is ours, prescription fulfilment is the natural, higher-margin extension, added when density supports it, not on day one.

A doctor, at your door.

On-demand doctors, the way ride-hailing moved cars. Choose the nearest available, affordable doctor, then get a real visit or an appointment.

We fill doctors' idle hours and strip out clinic overhead, so a doctor **earns more per hour** while a patient still **pays less than a walk-in**. Our take rate is the margin created in between, not a cost passed on.

1

Ready

Your history and records live with Iyashi.
Every doctor starts informed.

2

Choose

Sort by distance, price and rating.
Transparent fees, no surprise bills.

3

Visit

A home visit now, or an appointment for later. Care on your terms.

One tap, when it matters.

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THE MOMENT THAT MATTERS

05

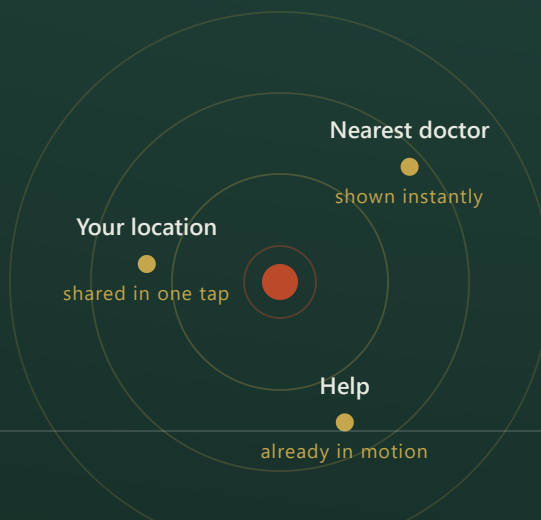
In a crisis you shouldn't be Googling a number. One tap shares your live location, shows the nearest help around you, and puts a doctor on our network on alert, so you act instead of panic.

4 apps to juggle → **1 button**

numbers to remember → **nothing to dial**

a stranger on video → **a doctor who knows you**

Iyashi coordinates and complements emergency care. It does not replace 108 / 112 or a hospital, and we never promise a guaranteed response time we cannot keep.



\$372B

TAM

India healthcare

\$107B

SAM

India digital health, 2033

\$5B

SOM

on-demand consult wedge

WHY NOW

25% a year

India's digital health market is compounding at ~25% annually. We enter at the highest-frequency moment, the doctor visit, then expand across the continuum of care.

BOTTOM-UP · CITY ONE

800 doctors × **90** consults/mo × **₹400** fee × **18%** take
= **₹6.2 Cr revenue** at full penetration in one city.

One engine now. Two more later.

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HOW WE MAKE MONEY

07

TAKE RATE · CORE · TODAY

Every consult and home visit

A transparent commission on each doctor interaction. High frequency, high margin, and it compounds as records deepen and patients return.

SUBSCRIPTION · LATER

Safety plans and B2B

Family safety plans plus societies, gyms and corporate contracts, a predictable second layer added once the consult loop is proven.

MARGIN · LATER

Delivery basket

Fulfilment margin on prescriptions, growing with consult volume, the natural capital-aware extension of the doctor relationship.

AVG CONSULT

₹400

OUR TAKE

₹72

CAC (TARGET)

₹180

CONTRIBUTION MARGIN

55%

Everyone owns a slice. No one owns the front door.

	EMERGENCY BUTTON	DOCTOR AT YOUR DOOR	MEDICINE DELIVERY	UNIFIED RECORDS	SINGLE FRONT DOOR
Practo	○	◐	○	◐	○
Apollo 24/7 · real hospitals	○	◐	●	●	○
Tata 1mg · delivery scale	○	○	●	◐	○
108 · Ziqitza	●	○	○	○	○
Iyashi	●	●	●	●	●

● Full capability ◐ Partial ○ None ● Iyashi, all five in one app

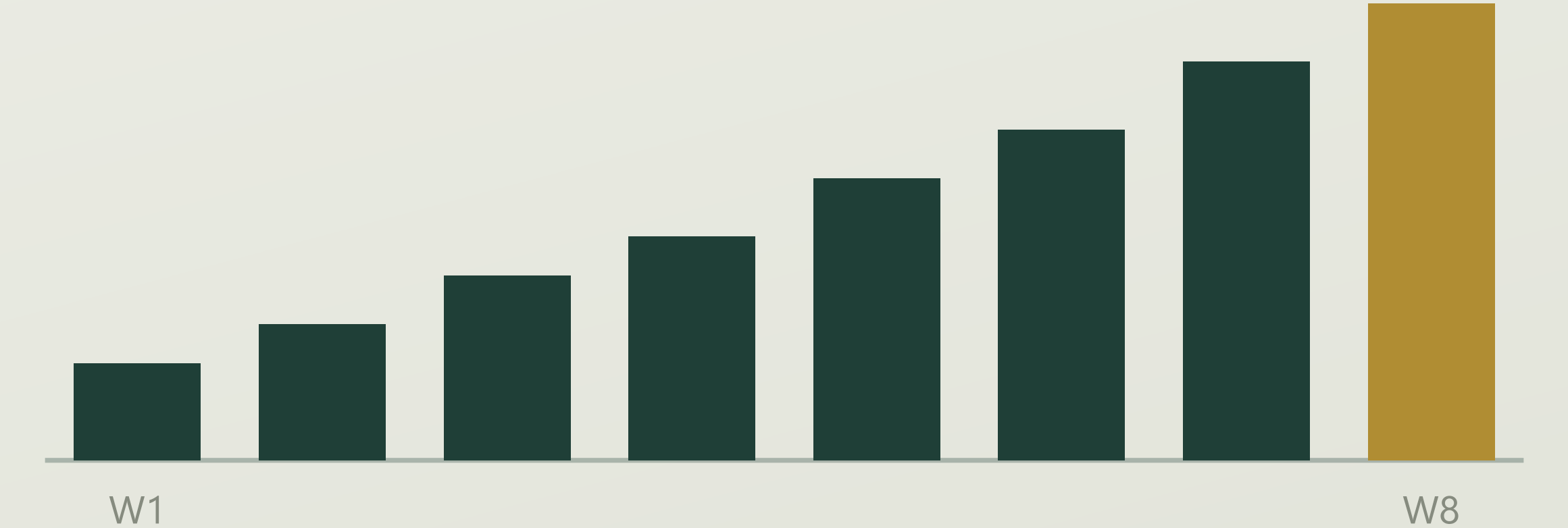
WHY THIS IS HARD TO COPY

Every incumbent is built around *its own asset*: a hospital, a pharmacy, a call centre. Iyashi is built around the *patient*. Neighbourhood density, unified records and the front-door habit compound together; a button bolted onto a hospital app does not.

38%
booked a second visit

900
waitlist in one pincode

CONSULTS PER WEEK · PILOT



₹1.4L
GMV to date

the one gap that matters.

THE FOUNDERS

We have built and owned it end to end

A founding team that has taken products from the first line of code to live users, across **Tronocity Labs**, **Gigzs** and **Akhrot**. We own the whole stack, ship fast, and have built lean without outside capital so far. Now all-in on lyashi.

THE GAP WE ARE CLOSING FIRST

A clinical co-founder

In healthtech, someone must own care quality, prescribing standards and compliance. We are actively recruiting a **Medical Director and clinical co-founder** plus a small medical advisory board, and we are filling this seat **before** we scale, not after.

Medical Director

Clinical advisors

Ops lead

Tronocity Labs

AI receptionists shipped for real businesses.

Gigzs

A marketplace product taken to live users.

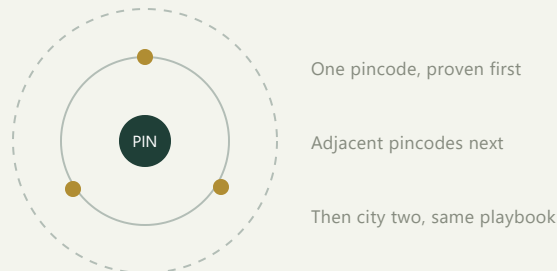
We have done the homework most skip.

Built to be compliant

- **Telemedicine Practice Guidelines, 2020:** registered doctors only; consults, prescriptions and records follow the RMP framework.
- **CDSCO and pharmacy rules:** prescription-required (Schedule H / H1) medicine dispensed only against a valid prescription, via licensed partners.
- **DPDP Act, 2023:** health data treated as sensitive, consent-first, encrypted, and portable via ABDM.

How we start, density first

Emergency, doctors and delivery all reward density. So we do not launch across India. We win one neighbourhood, then repeat the playbook.



Reachhead: [your launch pincode], the smallest area where doctors and patients are close enough for a reliable home visit.

Help us build India's front door to care.

12

RAISING

₹4 Cr

EQUITY

10%

RUNWAY

18 mo

This round takes us from 210 consults to a **repeatable city playbook**: one neighbourhood proven, the clinical seat filled, the loop ready to copy.

The Iyashi founding team · hello@iyashi.health

USE OF FUNDS, TIED TO MILESTONES

Doctor network and ops
to 800 active doctors in city one

40%

Product and engineering
records, booking, the button

30%

Growth and expansion
three pincodes proven

20%

Clinical and team
Medical Director hire

10%